

Name: Daniel S May | DOB: 10/11/1989 | MRN: 10105654130 | PCP: Joshua W Clark, NP | Legal Name:
Daniel S May

Office Visit - Sep 17, 2025

with Stacey Gray, MD at Mass Eye and Ear Sinus Center



Notes from Care Team



Progress Notes by Stacey Gray, MD at 9/17/2025 8:30 AM

Massachusetts Eye and Ear Infirmary

Department of Otolaryngology

Sinus Center

Stacey T Gray, MD

243 Charles St

Boston, MA 02114

Telephone: 617-573-4188

Pt name: Daniel S May

Date: 9/17/2025

MEEI #: 8026121

Progress Note:

I am seeing Mr. Daniel May in follow up for his history of chronic rhinosinusitis. To review his history from his first visit in August 2025, he is a 35 y.o. commercial real estate broker who presented to the Emergency Department in July 2025 with symptoms of congestion (all notes reviewed). He was recommended to use Flonase and follow up in Rhinology clinic. He returned to the Emergency Department in August (due to scheduling errors) at which time a Sinus CT was performed. This showed mild mucosal thickening throughout the paranasal sinuses, most prominent in the right maxillary sinus where secretions were also present. He told me that for about eight months he had issues with his nose and sinuses. Prior to this he did have frequent sinus infections (about six times per year) and ear blockage over the ten years prior. He also had two septoplasty procedures performed for nasal fractures during this time. He started using Navage at the onset of his symptoms. He added hydrogen peroxide to Navage. He was also using topical decongestant sprays frequently. He had improvement with this and had been able to clear out a lot of thick and sticky mucus from his sinuses. Sometimes it was yellow-green and other

times it was bloody. After starting Navage he could breathe better through his nose but he did have alternating congestion at night. He used oxymetazoline spray daily for this. He reported fluctuating sense of smell. He denied sinus pressure. He also felt like his ears were clogged. He reported popping and clicking at times. He felt he had thick mucus trapped in the back of his nose and sinuses that the oxymetazoline spray and Navage could not reach. He had no known history of seasonal allergies or asthma. He did have a Sinus CT in the ER earlier this week which showed mild mucosal thickening in the paranasal sinuses, most prominent in the right maxillary sinus where secretions were also noted (Dr. Gray interpretation). After his initial evaluation, we reviewed stopping Oxymetazoline and a prednisone taper was prescribed followed by Budesonide irrigations. Given his ear symptoms, Otology evaluation was also recommended.

At today's visit (9/17/2025) he presents for follow up. He reports his nasal congestion has improved. He is breathing well through his nose. He denies nasal drainage and post nasal drip. No sinus pressure. He is using Budesonide irrigations twice daily and has found these helpful. He is using decongestant nasal sprays less but still about 1-2 times per week. He reports persistent right ear blockage and crackling. He is scheduled for Otology evaluation in December. He does have some thick and clear mucus in his irrigations at times. Otherwise, he is doing well.

I, Elizabeth Passemato PA-C, saw the patient and performed the above elements of the history.

Physical Examination:

General appearance is well, with no signs of distress. Facial function is intact and symmetric. Eyes: No periorbital edema, extraocular movements intact. Ears: External ears are healthy. Anterior Rhinoscopy: No lesions noted. See nasal endoscopy for full details. Oral Cavity and Oropharynx: No erythema, no lesions, palate is symmetric, tongue is midline and fully mobile, no discolored drainage from the nasopharynx. Respiratory: Breathing comfortably, with no stertor or stridor. Skin: No vascular lesions or rashes on the face or the neck. Neurologic: CN3-7 grossly intact. Psychiatric: Appropriate affect. Lymphatic: No lymphadenopathy.

As a result of the patient's sinonasal symptoms which could not be adequately evaluated by external exam or anterior rhinoscopy, the decision was made to perform a sinonasal endoscopy. We discussed the risks, benefits, options, and complications of this procedure. We discussed the

risks of bleeding, pain, infection, limited views, and failure to meet the goals of the procedure.

Procedure:

After obtaining verbal consent, the nasal cavity was topically anesthetized using a combination of Lidocaine 2% and Oxmetazoline 0.025%. A zero degree endoscope attached to an HD camera was then introduced into bilateral nasal cavities. The interior of bilateral nasal cavities, middle and superior meati, the turbinates, and the spheno-ethmoidal recesses were examined. The video feed was projected onto the wall monitor in front of the patient and available for the patient to follow along.

Nasal cavity: Overall much less edema.

Inferior turbinates: Edematous bilaterally, more on the left, improved compared to last examination. No mucus present in the nasal cavity.

Middle turbinates and middle meatus: There is one clump of mucus along the face of the right middle turbinate. No mucus or inflammation in the middle meatus on either side. No polypoid change.

Superior turbinates and spheno-ethmoid recess: No polypoid change or mucus.

Nasopharynx: No mucus or inflammation.

Impression:

1. Rhinitis medicamentosa - improved.
2. Chronic rhinosinusitis.
3. Inferior turbinate hypertrophy.

Plan:

Mr. May and I reviewed his history and symptoms and the findings on nasal endoscopy. Overall his nasal symptoms have improved since stopping the hydrogen peroxide in his rinses and minimizing the Afrin and starting the Budesonide irrigations. I stressed the importance of complete cessation of all topical nasal decongestant sprays. He can continue the Budesonide irrigations and can try to decrease to daily. I do think that allergy evaluation will be useful for him and we will help arrange this. Most of his symptoms are currently ear related and we will try to help expedite his Comprehensive ORL evaluation for this. I asked him to schedule a follow up in 3-6 months for repeat examination. I asked him to call with any questions or concerns in the interim. All questions and concerns were answered.

Medication Review:

The risks, benefits, and side effects of prescription drug management with

topical steroid irrigations were reviewed and/or made available to the patient including the fact that they are not FDA approved and are being prescribed off-label. As such they may carry the same potential risks of oral steroids including Cataracts, Hypertension, Fluid Retention, High Blood Sugar, Diabetes, Osteoporosis, Gastritis, Avascular Necrosis, Immunosuppression, Skin Changes, Weight Gain, Adrenal Gland Suppression. The patient was counseled on the need for surveillance of these potential side effects including the need for prophylactic eye exams while on topical steroids.

I personally saw and evaluated the patient with Elizabeth Passemato, PA-C. I performed the history, examination, nasal endoscopy and impression and plan myself. I personally made all medical and surgical diagnoses and decisions for the patient. I have reviewed the note in it's entirety and composed/edited before signing.

Stacey T Gray, MD

Division of Rhinology

Department of Otolaryngology

Massachusetts Eye and Ear Infirmary

CC:

Pcp, Not Required

Unknown, Unknown, MD